

BUNDIBUGYO VIRUS DISEASE OUTBREAK

Democratic Republic of the Congo | Uganda | France

Weekly External Situation Report 13, Data as of 09 August 2026

Key Figures at a Glance (Democratic Republic of the Congo)

5 Provinces	4 Provinces	869	67.0%	16 183
53 Health Zones Affected	44 Health Zones Active Transmission*	Cases Recovered	Bed Occupancy	Contacts to follow
704 Active Cases in Isolation	155 Health Worker Cases	45 Health Worker Deaths	1 050 Bed Capacity	19 Testing Laboratories

Note: Health zones with active transmission have reported at least one case in the last 21 days.

Summary

Country	New Confirmed Cases (Last 7 days)	New Confirmed Deaths (Last 7 days)	Confirmed Cases (total)	Confirmed Deaths (total)	Probable Cases	Probable Deaths	Total Cases (Confirmed & Probable)	Total Deaths (Confirmed & Probable)	CFR (%) Confirmed & Probable
Democratic Republic of the Congo	579	304	4 381	2 011	-	-	4 381	2 011	45.9%
Uganda	0	0	20	2	1	1	21	3	14.3%
France	0	0	1	0	0	0	1	0	0.0%

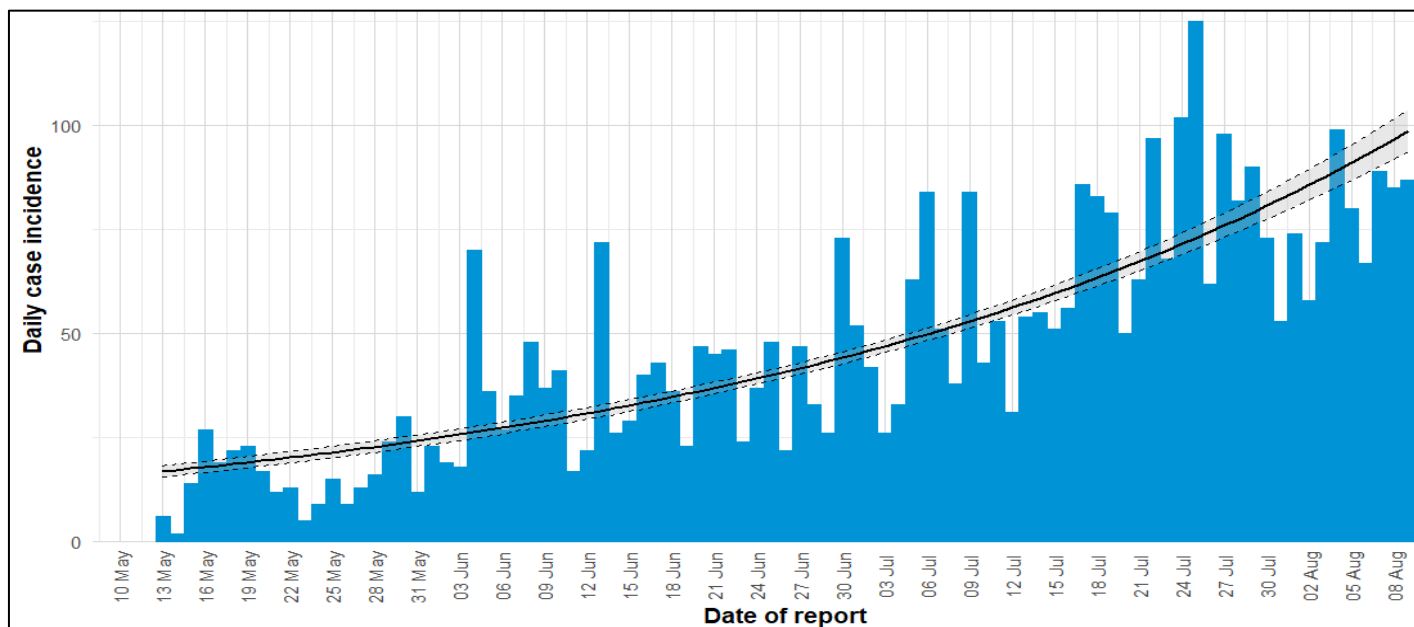
Note: Only confirmed and probable cases are reported.

Event description

Democratic Republic of the Congo

The Bundibugyo virus disease (BVD) outbreak in the Democratic Republic of the Congo remains in a phase of intense and expanding transmission, with cumulative deaths crossing the grim milestone of 2,000 only 86 days after the outbreak was officially declared on 15 May 2026. Since [External Situation Report #12](#), an additional 579 confirmed cases and 304 confirmed deaths have been reported, bringing the cumulative total to 4,381 confirmed cases and 2,011 confirmed deaths as of 9 August 2026, corresponding to a case fatality ratio of 45.9%. Ituri remains the epicentre, accounting for 85.8% of cumulative cases and 80.6% of cumulative deaths.

Figure 1. Daily log-linear growth trend of confirmed cases of Bundibugyo virus disease in the Democratic Republic of the Congo by date of report, data as 09 August 2026



The epidemic is increasingly characterized by sustained transmission within interconnected geographic clusters alongside continued geographic expansion. The Bunia–Rwampara–Mongbwalu–Nizi corridor remains the principal transmission focus, while persistent transmission in North Kivu and increasing activity in Haut-Uélé indicate spread beyond the original Ituri epicentre. The number of affected health zones increased from 51 in External Situation Report #12 to 53 as of 9 August 2026, with Gombari in Haut-Uélé and Bafwasende in Tshopo being the most recently affected health zones. Overall, the outbreak now affects 53 of 140 health zones across five provinces.

Figure 2. Map showing geographic expansion of the Bundibugyo virus disease outbreak in the Democratic Republic of the Congo, data as of 09 August 2026

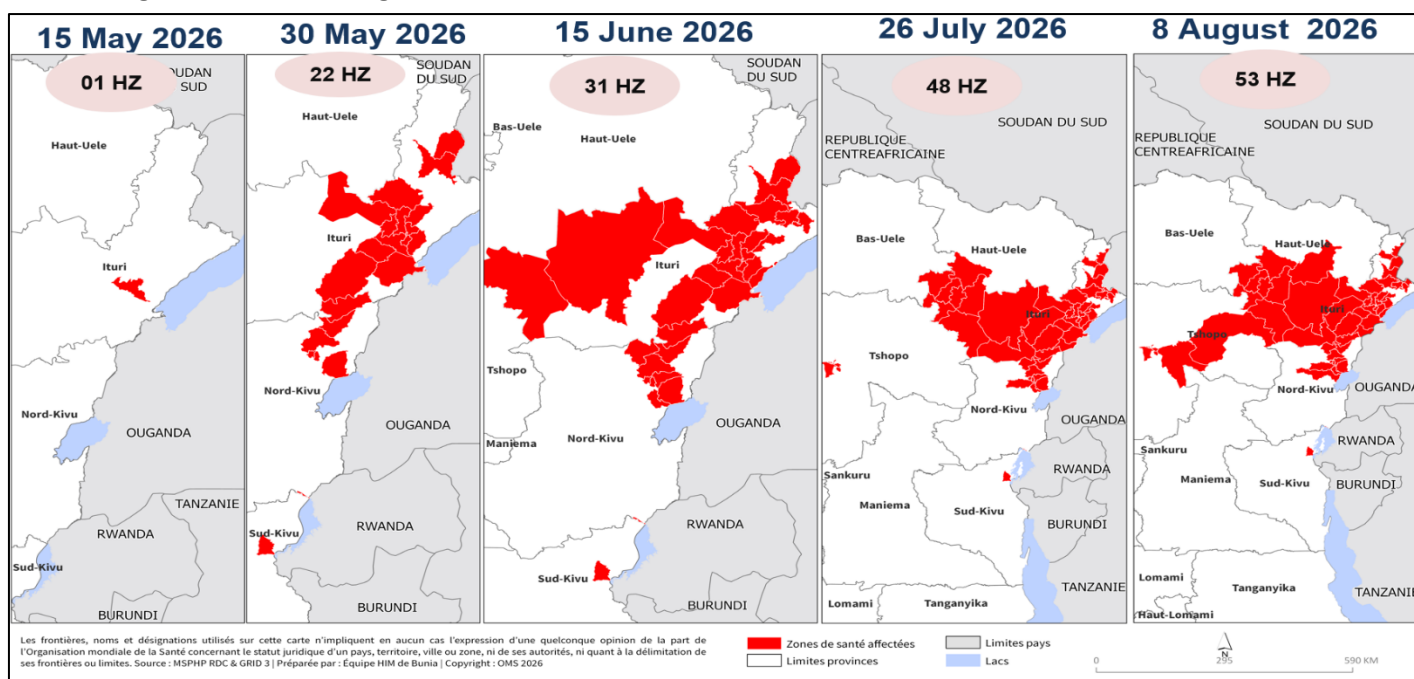
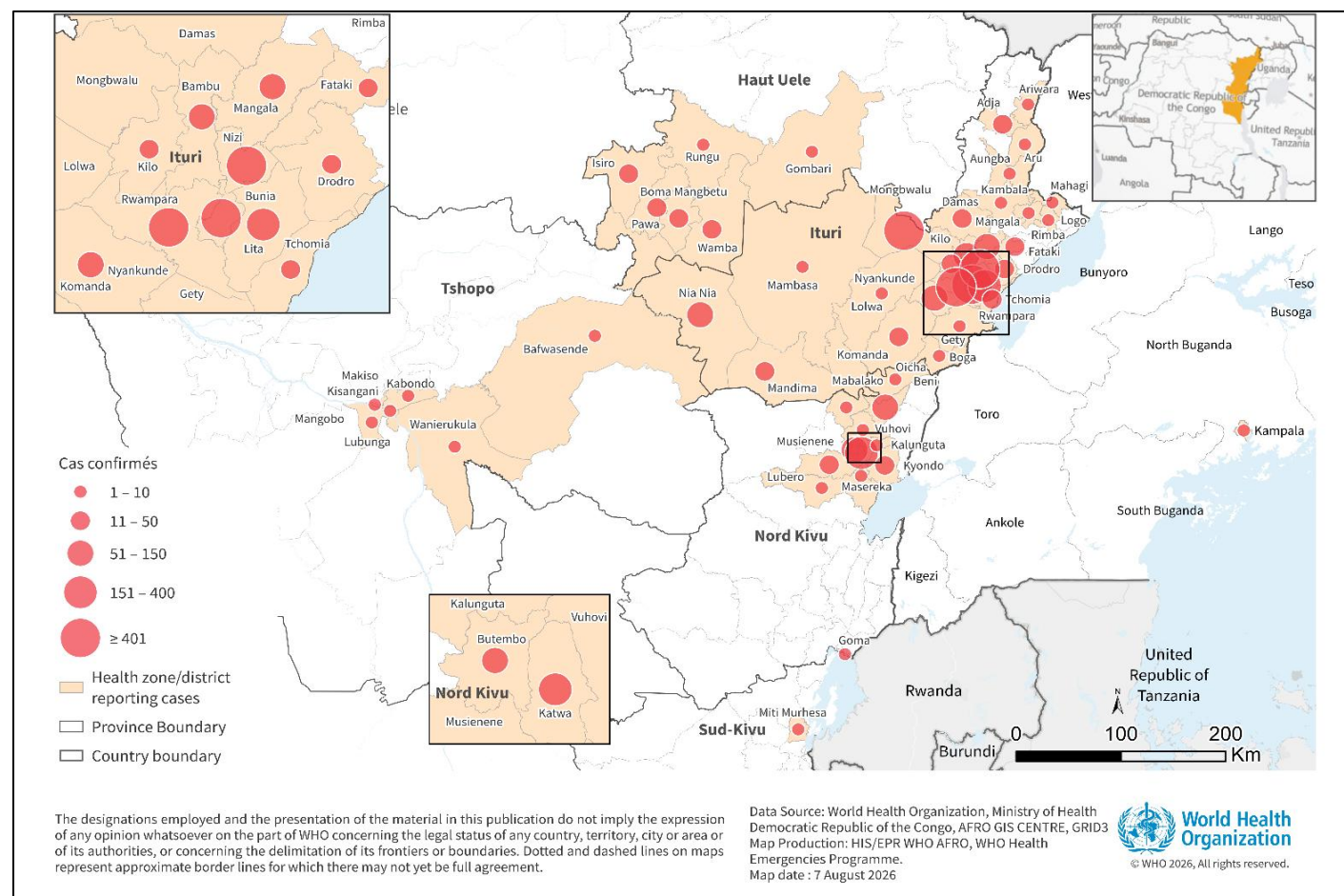


Figure 3. Geographical distribution of cumulative confirmed cases of Bundibugyo virus disease in the Democratic Republic of the Congo and Uganda, data as of 09 August 2026



Persistently high mortality continues to highlight gaps in early detection and timely access to care. Community deaths remain frequent, with 34 of 51 reported deaths (66.7%) occurring outside treatment centres on 9 August 2026. Surveillance performance improved, with all 242 validated suspected cases being investigated on 9 August 2026. Contact follow-up also increased to 86.7%, exceeding 85% for the first time, although performance remained substantially lower in Haut-Uélé at 57.9%, partly due to incomplete reporting. Despite these improvements, gaps in alert reporting, contact identification and follow-up around confirmed cases persist. Treatment capacity also remains under pressure, with several treatment and transit centres in Ituri saturated, while North Kivu continues to face constraints in referral capacity and Haut-Uélé still lacks a standard Ebola treatment centre in its six affected health zones.

Intense transmission is also occurring against a backdrop of increasing operational and workforce pressures. Challenges related to the timely remuneration of response personnel have been reported in some affected areas, with potential implications for workforce motivation and the continuity of response activities, including community-based interventions and operations at points of entry and points of control. Community resistance, insecurity and operational incidents continued to pose challenges to the timely implementation of response activities, including safe and dignified burials. These pressures are particularly concerning as epidemiological analyses indicate that transmission is occurring faster than cases are being detected and isolated, while contact-tracing capacity is increasingly stretched.

Despite the absence of further international transmission, the continued high incidence in eastern Democratic Republic

of the Congo poses a substantial risk of cross-border spread, particularly to Uganda and South Sudan through major population movement corridors. Surveillance at points of entry (PoEs) and points of control (PoCs) continued along key mobility corridors, although operational constraints persist at some sites. Continued strengthening of cross-border surveillance, information sharing and coordination with neighbouring countries remains essential for the early detection and management of potential cross-border transmission.

Uganda and France

No new BVD cases have been reported in Uganda. The last confirmed patient was discharged on 16 July 2026, and all identified contacts subsequently completed follow-up. As of 9 August 2026, 24 days had elapsed since the last patient's discharge without a new confirmed case. However, continued high transmission in neighbouring eastern Democratic Republic of the Congo poses a risk of reintroduction.

France has reported no secondary transmission following the imported case detected on 24 June 2026. The patient recovered and was discharged on 4 July 2026 after two consecutive negative polymerase chain (PCR) test results, and all five identified flight contacts completed 21 days of follow-up without developing symptoms. As of 9 August 2026, 36 days had elapsed since the patient's discharge without an additional confirmed case being reported in France.

Public health response

Coordination

- The WHO–Africa Centres for Disease Control and Prevention (Africa CDC) Continental Incident Management Support Team (IMST), based in Kampala, Uganda, continued to coordinate implementation of the Joint Continental Preparedness and Response Plan, supporting strategic and operational alignment across affected and at-risk countries. The mechanism continued to facilitate joint planning, information sharing, partner coordination, and mobilization of technical and operational support, with particular attention to cross-border preparedness and response.
- High-level WHO and Africa CDC leadership missions to Uganda on 3 August 2026 and the Democratic Republic of the Congo from 4 August 2026 reinforced political and strategic oversight of the response. In Uganda, WHO Regional Director for Africa Professor Mohamed Janabi and Africa CDC leadership engaged national authorities on maintaining preparedness against re-importation, strengthening cross-border surveillance, and enhancing regional coordination. The mission subsequently continued to the Democratic Republic of the Congo, where senior WHO and Africa CDC leadership reviewed the rapidly evolving outbreak and response operations and called for further adaptation and scale-up of the response.
- In the Democratic Republic of the Congo, national and provincial coordination mechanisms remained fully activated, with increased emphasis on decentralizing and adapting operations to persistent community transmission. Coordination focused on strengthening community-centred active case finding, contact tracing, and timely access to care; cross-validating surveillance, laboratory, and planning data; and strengthening coordination at points of entry and points of control along high-risk population movement corridors.
- WHO continued to coordinate with Africa CDC and a broad network of operational partners to address priority response gaps and strengthen national capacities. The United Nations Children's Fund (UNICEF), International Organization for Migration (IOM), Médecins Sans Frontières (MSF), Alliance for International Medical Action (ALIMA), International Medical Corps (IMC), Samaritan's Purse, European Union (EU), Global Outbreak Alert and

Response Network (GOARN), Emergency Medical Teams (EMTs), Health Cluster, and other partners continued to provide technical assistance, expert deployments, surge capacity, laboratory support, logistics, and field operations in affected and at-risk areas.

Operations Support and Logistics (OSL)

- The total emergency supply pipeline increased slightly from US\$11.29 million to US\$11.35 million. The value of supplies delivered increased from US\$4.14 million (37%) to US\$4.26 million (38%), while supplies in transit increased from US\$2.52 million (22%) to US\$2.64 million (23%). Commodities not yet ready for shipment decreased from US\$4.64 million (41%) to US\$4.45 million (39%), indicating a modest improvement in pipeline readiness.
- An additional approximately US\$120,000 worth of supplies was delivered during the reporting period. Cumulative delivered cargo increased from 312.5 to 316.7 metric tonnes, while the latest reported delivered volume stands at 1,787.4 cubic metres.
- Delivery of critical commodities to the Ituri and Kivu hubs continued. Cumulative deliveries include 66,974 hooded coveralls, more than 5.53 million examination gloves, 158,800 Level 3 gowns, 261 hospital beds, 10,076 infrared thermometers and more than 23,800 PCR tests. In addition, 29,100 hooded coveralls and 2.19 million examination gloves are currently in transit.
- Personal protective equipment and infection prevention and control supplies remained the largest procurement category, increasing slightly from US\$5.99 million to US\$6.03 million. This was followed by medical equipment (US\$1.81 million), laboratory supplies (US\$1.18 million), logistics supplies (US\$1.02 million) and medicines (US\$0.89 million).
- The overall supply pipeline showed modest improvement during the reporting period, with the proportion of commodities not yet ready for shipment declining from 41% to 39%. Nevertheless, US\$7.09 million (62%) of the total pipeline remains either in transit or not yet ready for shipment, underscoring the need to accelerate procurement and delivery of critical supplies as response operations continue. International suppliers account for 81% (US\$9.30 million) of the total procurement value, with WHO logistics hubs supporting consolidation and onward delivery to affected areas.

Surveillance

- Surveillance activities continued across affected and at-risk areas, including active case finding, alert management and investigation, contact tracing, mortality surveillance, community-based surveillance, and surveillance at PoEs and PoCs.
- Reporting of alerts remained high. On 9 August 2026, 1,426 alerts were recorded and verified across the five affected provinces, of which 242 (16.9%) were validated as suspected cases. All 242 suspected cases (100%) were investigated, and 124 were transferred to Ebola treatment or transit centres for isolation and further assessment.
- Surveillance at PoEs and PoCs continued along major population movement corridors. On 9 August 2026, 106,546 travellers were screened, representing 98% screening coverage, and 20 alerts were detected through PoE/PoC surveillance. Detected alerts were referred for further investigation and appropriate management.

- Contact tracing improved but remained below the operational target of 95%. As of 9 August 2026, 16,183 contacts were under follow-up, of whom 14,029 (86.7%) were successfully seen. Follow-up performance remained substantially lower in Haut-Uélé at 57.9%, partly due to incomplete reporting.

Laboratory

- Diagnostic capacity continued to expand through decentralized testing. The network includes 19 operational laboratories and testing units, with additional laboratories being established in Rwampara, Fataki, Bambu-Mine, Boga and Mambasa health zones in Ituri. In Haut-Uélé, a diagnostic platform has been installed in Isiro and seven RadiOne platforms have been acquired.
- Laboratory testing remained high, with sustained positivity indicating continued active transmission. Positive samples continued to be detected predominantly in Ituri, with additional positive results in North Kivu and Haut-Uélé, consistent with ongoing transmission across multiple affected provinces.
- Reverse-transcription polymerase chain reaction (RT-PCR) and rapid molecular platforms remain the main diagnostic methods. WHO has listed the first molecular diagnostic specifically for Bundibugyo virus under its Emergency Use Listing, while RadiOne platforms can provide results in less than one hour. Training of laboratory personnel on diagnosis, biosafety, post-mortem sampling and safe specimen transport continued.
- Genomic surveillance is ongoing in the Democratic Republic of the Congo, with more than 400 Bundibugyo virus genomic sequences generated and shared through open-access databases to support monitoring of viral evolution and transmission dynamics.
- Operational challenges persist, including incomplete laboratory reporting, delays in sample transport and return of results, gaps in data integration, and limited decentralized testing capacity in some transmission areas.

Case Management

- Admissions remained high as transmission continued. As of 9 August 2026, 704 patients were in isolation and receiving care nationally, against 1,050 available beds, corresponding to an overall bed occupancy rate of 68.8%. Despite available capacity at the national level, several treatment facilities in Ituri and North Kivu provinces remained saturated, including the Rwampara, Institut Supérieur des Techniques Médicales (ISTM) and Nizi Ebola treatment centres and the Bambu transit centre, highlighting geographic disparities in bed availability and continued pressure on case management services.
- Clinical outcomes continued to improve, with cumulative recoveries reaching 869 by 9 August 2026. During the previous 24 hours, 18 patients were declared recovered after two negative Ebola test results, including 14 in Ituri and four in North Kivu.
- Treatment capacity was further expanded during the reporting week with the inauguration of a new 100-bed Ebola Treatment Centre in Rwankole, near Bunia, Ituri Province, on 4 August 2026. The facility, the largest Ebola treatment centre established since the start of the outbreak, is expected to strengthen patient isolation and clinical care and relieve pressure on existing treatment facilities in Ituri.
- Rehabilitation and operationalization of treatment and isolation facilities continued in Musienene and Vuhovi in North Kivu. Haut-Uélé continued to lack a standardized Ebola treatment centre across its six affected health

zones.

- Comprehensive in-care support and clinical workforce strengthening continued, including nutritional and psychosocial support for patients and caregivers, referral and ambulance services, and training in the management of critically ill patients and use of high-flow oxygen. WHO and partners, including Médecins Sans Frontières and the World Food Programme, continued to support clinical care, treatment-centre operations, nutrition and workforce capacity.
- Mental health and psychosocial support continued, reaching 24 newly confirmed cases and 343 caregivers in Ituri and North Kivu, alongside support for children, families and frontline workers.

Research and Development

- Clinical research on therapeutics continued to expand in the Democratic Republic of the Congo. The WHO-sponsored PARTNERS trial has now enrolled 100 participants, with three sites operational and two additional sites in the process of activation. The trial is evaluating MBP134, remdesivir and their combination for the treatment of BVD.
- The EBO-PEP study has enrolled 69 participants and is developing an operational plan for mobile teams to expand access and enrolment of eligible high-risk contacts. The study is evaluating obeldesivir as a post-exposure prophylaxis.
- The MBOTE natural history study has also started, further expanding evidence generation on the clinical characteristics and progression of BVD.
- Vaccine development has progressed to first-in-human evaluation. The University of Oxford launched a Phase I trial of the ChAdOx1 BDBV vaccine in the United Kingdom on 13 July 2026, targeting 50 healthy adults aged 18 – 55 years, with the first participant vaccinated on 24 July 2026. Approximately 620,000 candidate vaccine doses have been manufactured, including 4,000 investigational doses for the Phase I programme. Regulatory approval is awaited for initiation of an additional Oxford Phase I study in adults in Uganda.
- In parallel, Moderna's mRNA-1469 candidate has entered Phase I evaluation at three sites in Canada, with the first doses administered to healthy volunteers as of 5 August 2026. The Vaccine Technical Advisory Group has also prioritized ERVEBO for inclusion as an arm in the planned Phase II/III BVD vaccine trials, with the protocol in the final stages of development.
- WHO and partners continued to advance the Bundibugyo medical-countermeasure research agenda, including development of vaccines, therapeutics and diagnostics. WHO has published a target product profile for Bundibugyo vaccines and regulatory considerations to support further development and potential emergency use.
- WHO and partners are also advancing a vaccine effectiveness evaluation to assess whether previous Ebola vaccination provides protection against BVD. The protocol is in the final stages of review, while work continues to compile, clean and harmonize historical vaccination data to support analysis.
- Operational research also continued in Ituri. Five protocols have received ethics approval, three of five planned studies have been completed, and four of 22 recommendations have been implemented. Current studies address laboratory-result turnaround times, decision-making processes, and social and behavioural factors, with findings being translated into response operations.

Infection Prevention and Control (IPC) and Water, Sanitation and Hygiene (WASH)

- IPC activities continued across affected health facilities and communities. In Ituri, 20 IPC rings were opened on 9 August 2026, and all 47 identified exposure sites were decontaminated. In North Kivu, 45 health facilities received IPC support, 168 health workers were briefed, and two facilities underwent IPC scorecard assessments.
- Safe and dignified burials (SDBs) continued across affected areas, although community resistance remained a challenge in some locations. On 9 August, 68 SDBs were conducted following 80 death alerts in Ituri (85.0%), and 50 bodies were sampled post-mortem. In North Kivu, 37 SDBs were conducted and 38 bodies were secured for post-mortem sampling.
- Healthcare worker infections remained a major concern, with 155 infections, including 45 deaths reported among frontline healthcare workers, underscoring persistent occupational exposure and the need to strengthen adherence to IPC measures in health facilities.
- Training, orientation and supportive supervision continued to strengthen frontline IPC capacity. In North Kivu, 453 health workers received IPC orientation, while facility assessments, supportive supervision, decontamination and household-level IPC interventions continued across affected areas. However, incomplete reporting and gaps in IPC implementation remain important operational challenges.

Risk Communication and Community Engagement (RCCE)

- RCCE activities intensified across affected provinces, with a focus on early care-seeking, contact tracing, safe and dignified burials and community acceptance of response measures. In Ituri, interpersonal communication addressed risks associated with caregivers, while community ambassadors were briefed in Mongbwali.
- Targeted engagement expanded among high-risk and mobile populations. In North Kivu, 20 motorcycle-taxi riders were briefed, 16 rumours were clarified, one community incident was resolved, and 103 household heads around 21 cases were reached. In Haut-Uélé, 1,231 people were reached through 351 household visits in Isiro, while 30 motorcycle-taxi drivers were sensitized in Boma-Mangbetu.
- Community feedback and trusted local networks continued to shape response interventions. WHO emphasizes engagement with traditional and religious leaders, women's and youth groups, community volunteers and other trusted actors to identify concerns, counter misinformation and adapt interventions to local realities. Recent reporting also indicates that community dialogue has improved demand for investigation, safe burials, care and reliable information in affected areas
- A second round of rapid community assessments has been initiated in two health zones, using interviews and focus group discussions to identify community perceptions, concerns and barriers to response interventions.
- Persistent mistrust and operational constraints continue to affect community acceptance. Denial of Ebola, resistance to safe and dignified burials, expectations regarding vaccine availability and non-payment of community actors remain important challenges, particularly in North Kivu and Haut-Uélé. WHO continues to emphasize large-scale trust-building, integration of community feedback and engagement of trusted local leaders as essential to improving adherence to response measures.

Preventing and Responding to Sexual Exploitation, Abuse, and Harassment (PRSEAH)

- PRSEAH awareness and capacity-building activities continued across affected provinces. In Ituri, North Kivu, South Kivu and Tshopo, 810 people, including 279 women, participated in orientation sessions targeting community actors, health care providers, rapid response teams, Provincial Health Division management teams, religious leaders, surveillance teams and hotel personnel.
- Community awareness activities reached 1,673 people, including 662 women and 170 girls, through targeted sessions for students, vendors, taxi and motorcycle-taxi drivers, health care providers and community leaders.
- A three-day government-led workshop was conducted to update the national PRSEAH response plan, bringing together key institutional stakeholders and technical partners to harmonize strategies, strengthen coordination and update operational procedures for PRSEAH across the outbreak response.
- PRSEAH capacity-building continued for community outreach workers, volunteers and focal points within rapid response teams. In addition, 18 WHO staff members received PRSEAH induction briefings and signed the WHO Code of Conduct.

Public health preparedness

Operational Readiness

- Regional preparedness continued to improve during the reporting period. A total of 31 Member States have completed and submitted their readiness assessment, increasing coverage to 66%. The average readiness score among Priority 1 and 2 countries remained at 58%.
- A total of six high-priority countries have completed repeat readiness assessments, demonstrating continued improvements in national preparedness capacities. These include Angola, South Sudan, Tanzania, Kenya, Rwanda, and Zambia.
- Regional deployments to strengthen preparedness continued in high-risk Member States. These include Burundi, Central African Republic, Democratic Republic of the Congo, and South Sudan.

Border Health, Travel and Mass Gatherings

- Screening and public health measures have been reinforced at airports, high-risk border crossings, and internal transit routes in both the Democratic Republic of the Congo and Uganda. National authorities, the International Organization for Migration (IOM), and partners continue to support the implementation of WHO border health guidance, strengthen cross-border coordination, and facilitate the timely detection, reporting and referral of suspected cases at PoEs and PoCs along priority travel corridors.
- Cross-border preparedness and response are being strengthened through enhanced coordination among the Democratic Republic of the Congo, Uganda, and South Sudan to improve the early detection of cases and reduce the risk of cross-border spread. Priority actions include strengthening coordinated cross-border disease surveillance, event-based surveillance, active case finding, joint investigations and contact tracing where

required, and real-time information sharing through regular cross-border coordination meetings and harmonized reporting mechanisms.

- Priority operational actions include rapid assessments and mapping of PoEs, strengthening surveillance along high-risk cross-border mobility corridors, including informal crossings, and major congregation points, and ensuring readiness capacities through the provision of trained personnel, equipment, PPE, referral mechanisms, disinfectants, risk communication materials and contingency plans for the safe management of suspected cases
- Mass gathering risk assessment capacity is being strengthened through training and technical support. The postponement of mass gatherings is advised in areas with documented BVD transmission until transmission is interrupted, while a risk-based approach is recommended for events held elsewhere in the region.
- Event-specific risk assessments should be conducted using the WHO all-hazards mass gathering risk assessment tool. These assessments should be complemented by implementation of measures before, during, and after events, as outlined in WHO guidance for mass gatherings attended by individuals from areas with documented Bundibugyo virus detection, including the establishment of referral pathways and RT-PCR testing capacity.
- Event organisers should distribute participant information to all attendees before and during the event, covering BVD symptoms, what to do if feeling unwell, and available health services, in line with [WHO guidance for mass gathering events attended by individuals from areas with documented BDBV detection](#).

Situation interpretation

The BVD outbreak remains uncontrolled, with transmission continuing to outpace response capacity. Persistent community deaths, geographic expansion and gaps in contact follow-up indicate continued undetected community transmission, while pressure on treatment facilities, uneven infection prevention and control capacity, workforce constraints and community resistance continue to challenge response effectiveness. The response should now shift to targeted interruption of transmission in the main clusters and emerging hotspots, while deepening community leadership and ownership of the response. This requires empowering trusted local leaders and community networks to drive active case finding, contact tracing, early care-seeking, and safe and dignified burials, alongside faster case investigation and isolation and rapid infection prevention and control interventions. Workforce and payment constraints require urgent resolution, while neighbouring countries should maintain heightened preparedness given the continued risk of cross-border spread.

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